

Project Magazine

Vol 4.



Project Closure Edition · Special Report

RESTORING HOPE, NORTHWest LIVES

How an Integrated Emergency Health and Nutrition Response reached thousands of displaced and host community members across Sokoto State and the human stories behind every data point.

Inside This Story

A Child Saved, A Mother's hope

Restored. Seven-year-old Basiru could not walk. Traditional healers had failed. One clinic visit changed everything for a family in Goronyo.



A publication of Life Helpers Initiative (LHI) · Funded by the Office of Humanitarian Assistance (OHA) / United States Government

By the

Number

2,367 Skilled Deliveries, Double the Target

A target of 1,169 deliveries assisted by skilled attendants was nearly doubled. The data tells a story of demand, trust, and capacity.



Nutrition in

Focus

How Ahmad Beat Malnutrition in 12 Weeks

Displaced from Kunawa, two-year-old Ahmad's MUAC moved from yellow to green and his mother found a new confidence in feeding her child.

Table of Contents

Cover Page (Front)	1
Table of Contents	2
Project Overview	3
Impact at a Glance	5
Humanitarian Approach	6
Health Sector	7
Nutrition Sector	9
Human Stories	10
Community Voices	11
Challenges & Response	12
Monitoring, Evaluation & Learning	14
Sustainability & Exit Strategy	15
Photo Speak	16
Partners & Stakeholders	18
Acknowledgements & Closing	19
Cover Page (Back)	20

A Consortium Built for Crisis-Affected Communities



The OHA-funded project (formally BHA) “Integrated Emergency Response to the Displaced Population in Zamfara, Sokoto and Katsina States, NW Nigeria” (Agreement No. 720BHA23GROO216) ran from **May 22, 2023 to June 30, 2026** under a consortium led by the International Rescue Committee (IRC) and comprising ALIMA, COOPI, Life Helpers Initiative (LHI), and GISCOR. Across the consortium's 11 LGAs in Zamfara, Sokoto and Katsina States, the project delivered integrated humanitarian assistance in Health, Nutrition, WASH, Protection, and Multipurpose Cash Assistance (MPCA) to conflict-affected, displaced, and host-community populations.



Within this consortium, Life Helpers Initiative (LHI) served as the Health and Nutrition lead for Sokoto State, delivering services across Goronyo, Rabah, Isa, and Sabon Birni LGAs for the full implementation period.

LHI Target Beneficiaries – Life of Award

Sector	People Targeted (Female, Male, Girls, Boys)
Health	255,655 total – 61,357 women · 40,905 men · 92,036 girls · 61,357 boys
of which IDPs	98,911 – 23,739 women · 15,826 men · 35,607 girls · 23,739 boys
Nutrition	223,055 total – 139,384 women · 17,352 men · 34,068 girls · 32,251 boys
of which IDPs	45,101 – 29,842 women · 15,259 men

Consortium-Wide Reach (Life of Award, all partners, all sectors)

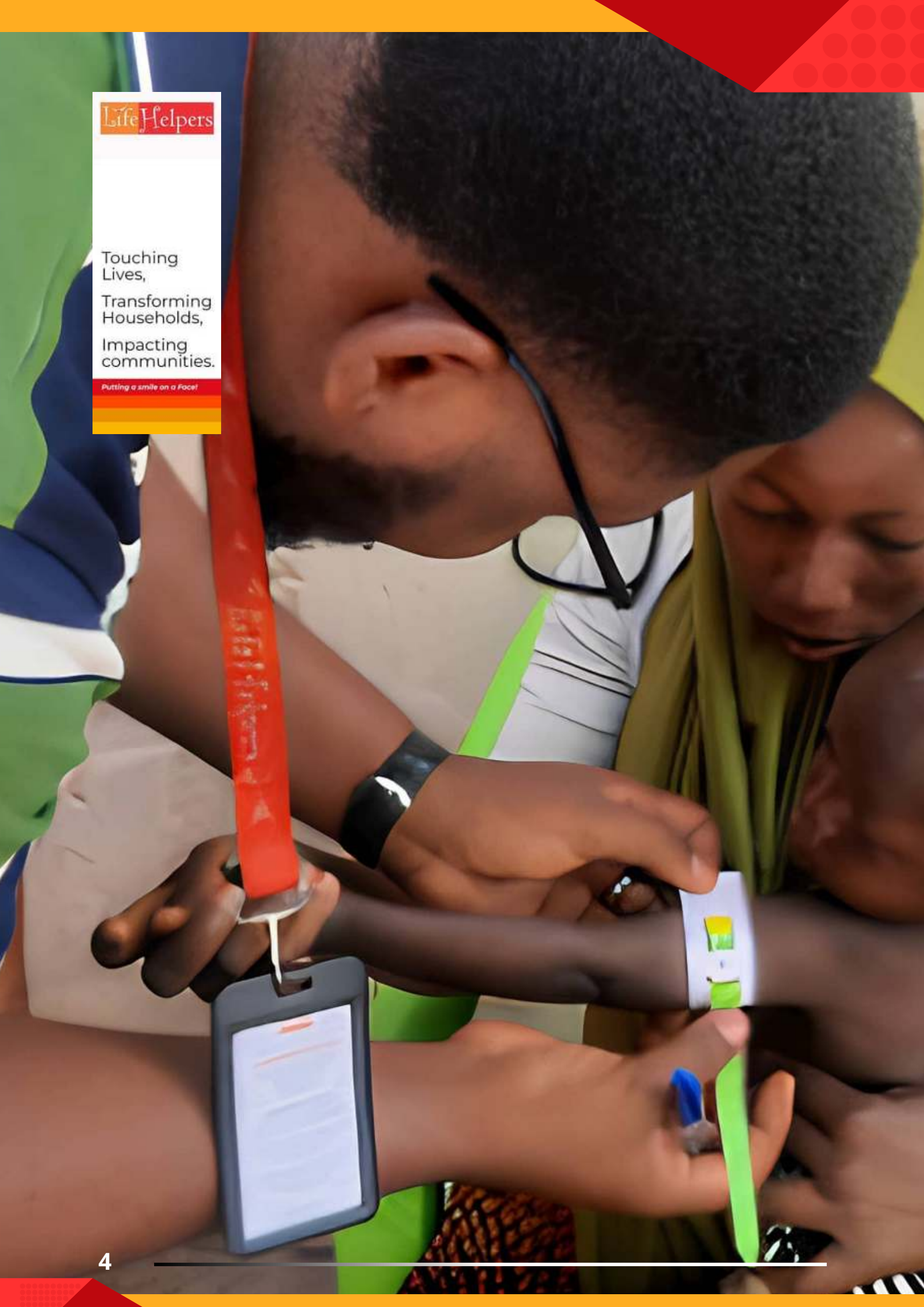
Population	Reached (LOA)
IDPs	1,202,532 (734,985 female · 467,547 male)
Host communities	647,517 (395,761 female · 251,756 male)
Total	1,850,049 people reached (1,130,746 female · 719,303 male)

Source: IRC Nigeria BHA/OHA Final Report (720BHA23GR00216), Health and Nutrition sector target tables.

Conclusion

Over three years of implementation in a highly volatile security environment, LHI's integrated health and nutrition programming significantly improved access to primary healthcare and nutrition services, strengthened community resilience, and through deliberate investment in government ownership, workforce capacity, and community structures left a sustainable foundation for continued quality healthcare delivery in Sokoto State beyond project closeout.





LifeHelpers

Touching
Lives,
Transforming
Households,
Impacting
communities.

Putting a smile on a Face!

The Numbers That Define This Response

From **May 2023 to June 2026** the full life of the award, LHI delivered the following cumulative, life saving results across its 10 supported health facilities in four Sokoto State LGAs.



Responding to Humanitarian Needs

Two integrated service pillars; **health and nutrition**, built to meet displaced and host communities exactly where the need is greatest, from the consultation room to the community compound.



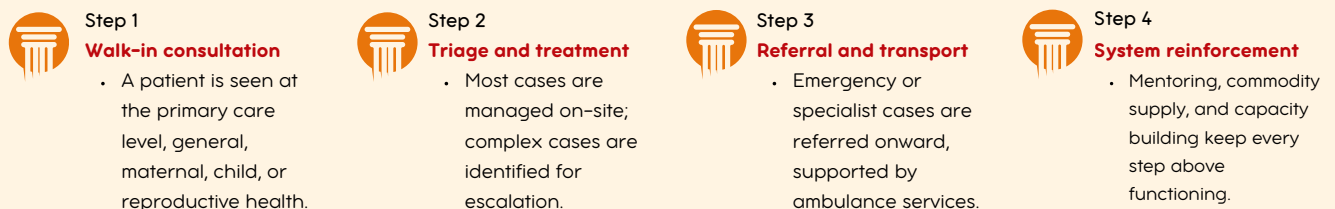
Health Services

A Comprehensive Package of Essential Care

The project delivered care across three connected pillars – from first contact at the clinic door to the systems that keep that clinic running.



How the Pillars Connect



Nutrition Services

Preventing and Treating Acute Malnutrition

Key Nutrition Interventions		
Treatment of Severe Acute Malnutrition (SAM)	Community screening	Family MUAC approach
Infant and Young Child Feeding support	Food demonstrations	Tom Brown preparation sessions
Defaulter tracing	Mother-to-Mother Support Groups	Men-to-Men Support Groups

The nutrition response combined clinical treatment with community-based prevention, reaching children early, and equipping caregivers to sustain progress long after a child is discharged.

Why Family MUAC Matters

Training caregivers to measure their own child's Mid-Upper Arm Circumference puts early detection directly in the hands of families, closing the gap between a child slipping into malnutrition and a caregiver knowing it's time to act.

Primary Care as a Lifeline

In communities where the nearest referral hospital can be hours away, the quality of primary care is the difference between life and death. Over the life of the award, LHI's health programming anchored a system of care for both IDPs and host communities across 10 supported facilities in Goronyo, Rabah, Isa, and Sabon Birni LGAs



Essential Healthcare Services

Outpatient · Maternal · Reproductive · Mental Health

- **109,225** outpatient consultations recorded – **86.5% (94,477)** new cases, **13.5% (14,748)** follow-up
- **93,159** communicable disease consultations (**85.3%** of caseload), including **54,671** confirmed malaria cases, **17,492** diarrheal disease cases, and **10,443** acute respiratory infections
- **38,250** non-communicable disease consultations, including **4,622** hypertension, **816** diabetes, and **965** chronic respiratory disease cases
- **9,550** pregnant women completed **≥2** antenatal care visits; Mama Kits distributed to encourage ANC attendance and facility-based delivery
- **5,635** safe deliveries conducted by skilled birth attendants
- **6,520** newborns (**3,591** girls, **2,929** boys) received postnatal care within 3 days of birth
- **1,222** people (1,073 female, 149 male) accessed Mental Health and Psychosocial Support (MHPSS) services
- **54** adolescent mentors (27 female, 27 male) delivered peer-led life-skills sessions to cohorts of ~15 adolescents



Health System Strengthening

Referral · Emergency · Pharma Upgrade

- **1,253** patients admitted to secondary/tertiary referral hospitals; 613 emergency referrals (398 female, 215 male) successfully completed
- Referral network of **4** secondary hospitals (Isa, Rabah, Goronyo, Sabon Birni) and 2 tertiary institutions (Federal Neuropsychiatric Hospital, Kware; UDUTH Sokoto)
- Financial support for approved emergency referrals to remove cost barriers for vulnerable households
- **7** primary healthcare facilities renovated across Goronyo, Rabah, and Isa LGAs; 3 mobile clinics established for Maitandu, 120 Rahama Estate, and 400 Housing Estate IDP Camp
- **58** healthcare workers completed competency-based training; 57 additional personnel received on-the-job mentoring
- **4** LGA M&E Officers, 10 Health Records Officers, and 41 frontline health workers trained in HMIS, CommCare, and data quality
- Pharmaceutical warehouse upgraded with a digital Inventory Tracking System (ITS) and bi-weekly commodity distribution, developed in capacity-sharing partnership with IRC

"Word-of-mouth referrals following successful treatment of complex cases such as uterine prolapse and septic wounds encouraged more community members to seek timely medical care
— IRC/LHI Final Report, Lessons Learned



LifeHelpers

Touching
Lives,
Transforming
Households,
Impacting
communities.

Putting a smile on a Face!

Fighting Malnutrition at the Root

Acute malnutrition is both a cause and consequence of displacement. Over the life of the award, LHI's nutrition programming expanded from an initial three sites in Goronyo and Rabah to ten sites across all four supported LGAs, combining clinical treatment with community-led behaviour change.

SAM TREATMENT AT LAJINGE & MAKUWANA OTP SITES

INDICATOR	RESULTS (LIFE OF AWARD)
Total SAM admissions	9,614 children (4,455 boys · 5,159 girls)
• Lajinge OTP	5,112 children (2,407 boys · 2,705 girls)
• Makuwana OTP	4,502 children (2,048 boys · 2,454 girls)
Recovered / discharged cured	7,698 children (3,431 boys · 4,267 girls)
Defaulted	298 children (71 boys · 227 girls)
Transferred to Stabilization Centre	46 children (24 boys · 22 girls) – complicated cases

Community Screening & Family MUAC

- 76,257 children aged 6–59 months screened for acute malnutrition (33,503 boys, 42,754 girls) through community and facility screening

- 47,656 MUAC tapes distributed to 47,565 caregivers (39,532 women, 8,033 men) trained to screen their own children at home

- 56 Community Nutrition Mobilizers (54 women, 2 men) led active case-finding, referral, food demonstrations, and defaulter tracing



Maternal, Infant and Young Child Nutrition (MIYCN)

- 63.4% of infants aged 0–5 months at supported facilities were exclusively breastfed
- 4,170 children aged 6–23 months met the Minimum Dietary Diversity (MDD) standard of ≥5 food groups
- 33,790 pregnant women received individualized MIYCN counselling
- 112 Mother Support Groups established, reaching 1,680 women
- 8 Men-to-Men Support Groups reaching 80 male participants
- 80 Role Model Mothers serving as community nutrition champions
- 16 bi-monthly food demonstration sessions, each reaching 120–150 caregivers

Tom Brown Programme for Moderate Acute Malnutrition (MAM)

- Introduced as a pilot to close a critical service gap – no partner or government programme was managing MAM in the intervention areas at project start – the Tom Brown programme enrolled 500 children (207 boys, 293 girls) with MAM across the four LGAs. Caregivers learned to prepare the traditional, locally sourced complementary food through weekly sessions, and children's Mid-Upper Arm Circumference improved by 0.3–0.6 cm on average during the intervention.

The Faces Behind the Data

Every statistic in this report represents a real person who came to a clinic, sat with a health worker, and left changed. These are one of those stories.



A Child Saved, A Mother's Hope Restored

In Goronyo, Nigeria, seven-year-old Basiru Lawal faced a life-threatening condition that left him unable to walk. After developing severe swelling in his leg, his family sought traditional treatment, but his condition worsened, spreading and causing intense pain.

His mother, Fatima, believed the illness was spiritual and had nearly lost hope. Through a community engagement supported by LHI a project supported by the USG, she was encouraged to seek medical care. At the LHI Goronyo IDP health Clinic, Basiru received comprehensive treatment, including wound care, antibiotics, nutrition, and rehabilitation.

Within weeks, his condition improved significantly. The swelling reduced, his strength returned, and he gradually began walking again soon running like other children.

"Na riga na yanke kauna, na yi tunanin ba zai sake tafiya ba."

— Fatima, Basiru's mother (translated: "I had already lost hope. I thought he would never walk again.")

Today, Basiru is healthy and active, fully returned to childhood life. This intervention not only saved a child's life but restored a mother's hope and trust in healthcare demonstrating the power of timely, life saving support.



Trust Built Through Consistent Care

Across the four LGAs, the strengthened referral system and consistent quality of care shifted health-seeking behaviour: families who initially relied on traditional medicine and community healers increasingly turned to the formal health system.

Fifty-four trained adolescent mentors extended this trust to young people, running peer-led life-skills sessions on sexual and reproductive health, GBV prevention, and psychosocial wellbeing, including a non-formal education centre established with the Sokoto State Emergency Management Agency (SEMA) inside the Goronyo IDP Camp.

Success Story · Nutrition

How Nutrition Support Saved Ahmad's Life

Two-year-old Ahmad Baduga was the third child in a family displaced from Kunawa to an IDP camp in Goronyo. A routine MUAC screening placed him in the yellow zone, Moderate Acute Malnutrition. His mother, struggling with limited resources and gaps in her knowledge of complementary feeding, could not provide what Ahmad needed on her own.

Through the USG-funded programme, Ahmad and his mother were enrolled in a 12-week nutrition programme within a caregivers' support group. She learned hands-on how to prepare Tom Brown, a locally nutritious complementary food, alongside hygiene practices and age-appropriate child feeding.

Week by week, she applied what she had learned. Week by week, Ahmad changed.

"Yanzu na koyi yadda zan hada abinci mai gina jiki ga dana."

— Ahmad's mother (translated: "Now I have learned how to prepare nutritious food for my child.")

Ahmad's MUAC moved from yellow zone to green, normal nutritional status



Operating Under Pressure

Delivering humanitarian health services in Northwest Nigeria requires navigating a constellation of acute operational risks. These are the four primary challenges and how the team responded.



Joint supportive supervision team conducting a facility review, 2026.

01 Heightened Insecurity and Continuous Displacement

Armed banditry and ongoing conflict caused repeated displacement of households mid-programme, fragmenting patient continuity and requiring mobile outreach teams to constantly reorient to population movement.

→ Mobile clinic services and community health worker networks maintained fluid coverage

02 Limited Physical Access Due to Distance and Transportation

Many community members, particularly women carrying young children, faced significant barriers reaching health facilities due to distance, lack of transport, and insecurity on roads connecting remote settlements to PHCs.

→ Emergency ambulance services and expanded community outreach reduced physical barriers

03 Stock-Outs of Essential Commodities and Drugs

Supply chain disruptions, compounded by the complexity of cross consortium coordination, led to periodic shortfalls in essential medicines and nutrition commodities, threatening continuity of care at critical moments.

→ Pharma warehouse upgrade with IRC improved inventory management and buffer stock tracking

04 No Water Supply at Two Health Facilities

Lajinge and the Goronyo mobile clinic operated without consistent water supply, a critical infection prevention and control gap that affected hygiene practices in maternity and treatment areas.

→ Water trucking and IPC adaptations maintained minimum standards pending infrastructure solutions

LifeHelpers

Touching
Lives,
Transforming
Households,
Impacting
communities.

Putting a smile on a Face!



What the Data Revealed About Itself

A commitment to honest self-assessment is the mark of a learning organisation. Over the life of the award, LHI invested in strengthening its Health Management Information System (HMIS) and translated field-level findings into concrete practice changes.



01 Capacity Building:

4 LGA M&E Officers, 10 Health Records Officers, and 41 frontline healthcare workers trained in HMIS, CommCare digital reporting, and data management

02 Data Quality Assurance:

Routine Data Quality Assessments (RDQAs), data verification exercises, and mentoring visits improved completeness, accuracy, consistency and timeliness of routine data

03 Joint Supervision:

Joint supportive supervision with SMOH, SSPHCDA, Local Government Health Authorities, and MEAL teams reviewed service delivery, IPC, referral systems, and information management

Key Lessons Learned

1. Adaptive programming is essential – flexible workplans and resource reallocation sustained services through insecurity, displacement and flooding.
2. Strong government leadership and multi-sectoral partnerships improve programme effectiveness and minimize duplication.
3. Community ownership and trust – built through outreach, sensitization and VCAT sessions – directly increase service utilization.
4. Leveraging trusted community structures, such as Traditional Birth Attendants, strengthens referral systems and reach.
5. Beneficiary feedback mechanisms drive continuous programme improvement and responsiveness.
6. Integrated services (health + nutrition + education/protection) generate broader development outcomes for adolescents and displaced households.
7. Institutionalizing joint supportive supervision, harmonized information systems, and early transition planning are essential for sustaining gains after donor support ends.

Building Systems That Outlast the Project

True impact is measured not only by what happens during a project, but by what remains after it closes. LHI's exit strategy was designed to leave stronger systems, more capable health workers, and engaged government ownership.



Health Worker Capacity Building

All health workers in supported facilities received structured training in IMCI (Integrated Management of Childhood Illness), IMAM (Integrated Management of Acute Malnutrition), BEmONC (Basic Emergency Obstetric and Neonatal Care), and CEmONC (Comprehensive EmONC). These skills belong to the workforce permanently.

1



Government Ownership & Handover

LHI engaged government stakeholders to take formal ownership of all supported facilities, including the mobile clinic. Staff were posted to the mobile clinic by the government authority prior to handover, ensuring uninterrupted service delivery rather than a cliff edge closure.

2



Asset Handover & Documentation

Formal asset handovers have been completed for Isah, Rabah PHC, and Rarah PHC. Reporting completion and remaining outstanding actions are actively being processed to ensure a clean, accountable project closure that satisfies donor requirements.

3



Supply Chain Systems Upgrade

The pharmaceutical warehouse upgrade achieved through collaboration with IRC represents a lasting institutional asset. Standardised supply chain practices now embedded in LHI's operations will benefit future emergency responses across the organisation's programming geographies.

4



Community-Level Knowledge Transfer

Over 10,463 caregivers trained in Family MUAC, 5,215 reached with MIYCN behaviour change, and a network of Mother-to-Mother support groups now active in communities each one a peer educator capable of sustaining nutrition knowledge beyond the life of the project.

5



State Coordination Integration

Monthly nutrition coordination meetings were held with the State throughout the project period, ensuring that LHI's data, learnings, and practices were embedded in the government coordination architecture not siloed in a parallel humanitarian system.

6

Photo Speaks



Photo Speaks



The Consortium That Made It Possible

No single organisation delivers a complex emergency response. Behind every clinic visit, every nutrition screening, and every skilled delivery stands a web of partners working in coordination.



Donor

Office of Humanitarian Assistance (OHA)

United States Government funding channelled through OHA enabled the full deployment of the integrated health and nutrition emergency response across all four LGAs in Sokoto State.



IRC Collaboration Highlight

Beyond consortium leadership, IRC provided direct technical capacity sharing support to LHI's pharmaceutical warehouse a partnership that produced lasting systems improvements that extend well beyond this project period.

Implementing Partners

Lead Consortium



Health & Nutrition Lead (Sokoto)



Consortium Partner (Protection)



Consortium Partner (WASH & Protection)



Health & Nutrition Lead (Katsina) Consortium Partner



Government Stakeholders

State Primary Healthcare Development Authorities in Sokoto, Zamfara, and Katsina served as the critical government interface for this response from facility level coordination to asset handover and the staffing of transitioned facilities post-closure.



SSPHCDA
Sokoto State



ZSPHCDA
Zamfara State



KSPHCDA
Katsina State

Acknowledgements & Closing

"This intervention not only saved a child's life but restored a mother's hope and trust in healthcare, demonstrating the power of timely, life-saving support."

—
LHI Field Documentation,
Goronyo IDP Health Clinic, 2025

Once again, Life Helpers appreciate and celebrate every partners and officers who have impacted us throughout the implementation of the OHA PROJECTS in Sokoto.

—
Tayo Fatinikun (FICA, FIMC, CMC)
National Executive Director (Life Helpers)

From the families who opened their doors to community health workers, to the nurses and midwives who stayed on after hours, to the government officers who sat with us through every coordination meeting, this project was built by people who refused to accept displacement as someone else's problem.

With Thanks To



Donors
OHA / United
States Government



Partners
IRC, COOPI, ALIMA,
GISCOR



LHI Staff
Clinicians, CHWs
& support teams



Government
SSPHCDA,
ZSPHCDA,
KSPHCDA



Communities
Goronyo, Rabah,
Sabon Birnin, Isa



Beneficiaries
IDPs & host
community
members

For any feedback, suggestion, Appreciation or complain please contact us on this

Toll Free Line this Number: 02013309033

Email Us on: feedback@lhinigeria.org

Radio Nigeria
(Royal F.M 101.5, Sokoto State)
Every Tuesday: 11:00AM - 12:00PM (GMT)



WESPEAK (MUYI MAGANA)

WeSpeak, also known as Muiy Magana, is a weekly radio program implemented by Life Helpers Initiative in Sokoto State and aired weekly by Radio Nigeria (Royal F.M 101.5). The program cuts across all thematic areas, health, education, livelihood, agriculture, and gender equity.

The platform provides timely information, encourages dialogue, and creates space for questions, feedback, and external complaints. Community members share concerns, receive referrals, and learn about available services. Civil society organizations are also invited to use the platform to raise awareness about their work and promote collaboration.

The radio program has touched the lives of many people across Sokoto State by increasing awareness, promoting positive behavior change, and strengthening accountability.

EDITORIAL TEAM

Tayo Fatinikun (FICA, FIMC, CMC)	Editor-in-Chief, the NED
Precious Afuaman	Deputy Editor, the D.MEL
Hauwau Budah	Project Manager, Sokoto
Ibrahim Badamosi	MEL/IT Specialist (Data)
Joy Dogo Dauda	Content Editor, Photographer
Oluwafemi Tanimowo	Visual Design / Lead Illustrator
Tola Olabode	Asst. Photographer

OUR MISSION

Be a leading non-governmental organisation working to maximise all opportunities to empower the marginalized people.

OUR VISION

A more Fulfilled Life for Everyone

